

Patient 18

Report A

6551519 O2f07/2016 CT Acute Stroke

For Thrombolysis? Not a thrombolysis call

Report:

Indication: Word finding difficulties. Workup at approximately 06:45 a.m...? Left MCA stroke. CT HEAD

Haemorrhage: No

Intravascular thrombus: NO

Acute infarct: There is a focal area of low attenuation involving the genu of the left internal capsule and medial left lentiform nucleus, which may represent an area of infarction.

Other findings and comments: There is mild dilatation of the lateral and third ventricles with no obstructive cause identified. The configuration of the third ventricle suggests that this is not acute. The basal cisterns remain patent. There is cerebral deep periventricular white matter low attenuation likely to represent the sequelae of mild chronic small vessel ischaemic change.

CT ANGIOGRAM

Window settings: (intraluminal HU=280; W=560; L=210)

(ICA Stenoses NASCET method, measurements on axial 1mm sections) Aortic arch and subclavian arteries: Non Stenotic Calcification in the arch.

R CCACA: Normal

L CCA/ICA: Normal

RVA: Normal

L VA: Focal calcification at the origin however the vessel is patent.

Intracranial Arteries: Patent vessels of the circle of Willis with no significant stenoses.

Other findings and comments: There is an enlarged pre-tracheal lymph node measuring 15mm x 15mm, Prominent but not significantly enlarged nodes are also seen in the aorto-pulmonary region, No pulmonary nodules or consolidation within the imaged segments of the upper lungs,

There is interlobular septal thickening, a finding seen with pulmonary venous congestion, clinical correlation is suggested. No focal bone lesion.

Conclusion:

No intracranial haemorrhage. Possible infarct involving the left internal capsule/lentiform nucleus. MRI is suggested if there is continued clinical Concern.

Moderate lateral ventricle and third ventricular dilatation with no obstructive Cause demonstrated. Solitary enlarged mediastinal lymph node of indeterminate aetiology. Correlation with the clinical history is suggested in the first instance with consideration for further imaging of the thorax.

Report B

6551593 04/07/2016 MR Head

Comparison with previous CT imaging 02/07/2016

Indication: Expressive aphasia ? left internal capsule/lentiform nucleus infarct.

Findings: There is diffusion restriction demonstrated in the left premotor cortex consistent with an acute left MCA infarct, on a background of periventricular and the white matter high T2 signal foci in keeping with mild chronic small vessel disease. No other acute parenchymal pathology is identified. Mature lacunar infarct in the left lentiform nucleus. No midline shift or mass effect. No acute intra or extra-axial haemorrhage. There is mucosal filling of the right maxillary antrum. No circle of Willis or proximal ACA, PCA or MCA aneurysm, stenosis or occlusion.

Conclusion: Acute left MCA infarct on a background of moderate chronic small vessel disease.